

Social Identity Theory

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In almost all contexts, people live in social groups, considering and describing themselves not only as unique individuals but also as members of certain social groups. Perceiving oneself as a social being provides one with a sense of social identity. Social identities can be based on race, age, gender, religion, employment, partisanship, and nationality, to name a few. In health communication, identities can be based on groups that are associated with certain conditions or behaviors (e.g., cancer survivors, smokers, runners). As such, social identity is a product of communicative behaviors, as it is formed, developed, and maintained in the process of ongoing social interactions (Scott, Corman, & Cheney, 1998). Through communication, individuals define themselves, express their belongingness to various collectives, assign values to their group image, and use their continually communicated identities to navigate their lives (Ashforth, Harrison, & Corley, 2008). Thus, it is evident that social identity is important to health communication as examining individuals' social aspects of life can contribute to a nuanced understanding of their health-related beliefs, attitudes, and behaviors. This entry provides an overview of social identity theory, explicates definitions and components of key constructs in the theory, and discusses the impact of social identity on health communication from the perspectives of group norms, social support, self-efficacy, and message effects.

Overview of social identity theory

Social identity theory suggests that people define the sense of self in terms of social categories and group memberships. Social identity refers to one's "knowledge that [they] belong to certain social groups together with some emotional and value significance to [them] of this group membership" (Tajfel, 1972, p. 292). In contrast, personal identity, which is the other part of self-concept, refers to one's perception of the unique characteristics and idiosyncratic attributes of self. Individuals categorize themselves and others into social groups and differentiate in-groups from out-groups, a process known as social categorization. When people identify with a group, they consider the

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members as “us” instead of “them” or “others.” One critical reason why people perceive social groups in this way is that group affiliation offers a source of security and companionship as well as a sense of purpose and worth (Haslam et al., 2009). Accordingly, people derive self-esteem from the groups they belong to and strive to maintain a positive image of the self and a favorable evaluation of the group. The social aspects of people’s lives not only define who they are, but also inform what they think, feel, and do, and explain why they approach and interact with others the way they do. In the context of health communication, social identity has the capacity for shaping attitudes, guiding behaviors, and influencing decision-making processes.

Components of social identification

Social identification refers to “the perception of oneness or belongingness to some human aggregate” (Ashforth & Mael, 1989, p. 21). In line with the conceptualization of social identity, the stage of identification involves multiple important components (Tajfel, 1982): (i) a *cognitive* component – an awareness of one’s group membership, entailing the possibility to distinguish one group from other social categories (e.g., I am a healthy eater); (ii) an *evaluative* component – a value connotation associated with the group, reflecting the group self-esteem (e.g., I value healthy eating; Having a healthy diet is important to me); and (iii) an *emotional* component – an attachment to the group, capturing a feeling of affective commitment (e.g., I feel proud of being a healthy eater). In addition, a broader scope of identification contains (iv) a *behavioral* component – an inclination to act in terms of one’s group membership (e.g., I maintain a healthy diet). In sum, the components of social identification involve cognition, emotion, and behavior; that is, one can think, feel, and act one’s way into identification (Ashforth et al., 2008).

Although these components are closely related, they do not always go together (Ellemers, Kortekaas, & Ouwerkerk, 1999). Imagine Amy, an employee of a company who also considers herself a vegetarian. To identify with the company, she must be a member of the company (i.e., group membership). This is because identification with a social group requires an individual to have an identity as a member of that group. It is important to note that even though Amy acknowledges that she is an employee affiliated with the company, she may or may not feel involved or identify with the company (i.e., affective commitment) or perform behaviors in terms of the group membership. Instead, she may prefer to identify with another group of which she has a salient identity as a member (psychologically central and subjectively important to one’s sense of self), such as a health-related group in which the members do not consume animal products, accentuating her identity as a vegetarian and leading to identity-based behaviors. In this sense, social identities are relational and comparative. As this example demonstrates, it is the extent to which people identify with a social group, as opposed to merely group membership, that determines whether they are likely to behave in terms of the group membership (Ellemers et al., 1999).

Social identity and health-related behaviors

As articulated above, the extent to which people identify with a particular group shapes their attitudes and behaviors in line with their group membership. In the context of health communication, when people strongly identify with a particular group, they are more likely to endorse health-related beliefs and engage in health-related behaviors or activities advocated by the identified group. The mechanisms through which social identity influences health-related behaviors include conformity to group norms, perceived social support, and enhanced self-efficacy.

First, social identity influences health-related behaviors through the role of group norms. Social identity facilitates the formulation of and conformity to group norms, which regulate thoughts, feelings, and behaviors of people in a social group and differentiate the in-group from out-groups. Through social categorization, people represent groups as prototypes, which are general attributes that depict one particular group, capturing intragroup similarities and intergroup differences (Hogg & Reid, 2006). Social categorization also depersonalizes people's self-perception as well as their perception of others in terms of in-group and out-group prototypes. That is, the uniqueness of each individual is submerged into a single representation that captures the prototypical attributes of the group. In this sense, the shared prototypes among people in one group (i.e., group prototypes) describe behaviors that are representations of group norms. More importantly, group prototypes also prescribe behaviors that inform people how they should behave in terms of group membership. Therefore, the more strongly one identifies with a social group, the more likely one conforms to in-group normative behaviors that have significant potential for influencing health-related decision making.

Second, the impact of social identity on health-related behaviors is through the function of social support. Social identity provides a basis for cultivating social support given, sought, and obtained among group members. Integration into social networks offers a sense of embeddedness in a social system that enables individuals to receive feedback from others, leading to feelings of stability, predictability, and control over their lives. The strength of one's social identity with a social group determines the extent to which one perceives and benefits from social support from that group. Moreover, the effectiveness and usefulness of social support depend on the match between the provider and the receiver of the support, such that people are more likely to provide and receive support when the provider and the receiver have an identity-based relationship. Identification with a group has been shown to buffer the adverse effects of stress and enhance life/job satisfaction through the mediating role of social support (Haslam et al., 2005). Given this, when people strongly identify with a social group, they are more likely to benefit from in-group social support, leading to positive mental and physical health outcomes.

Third, social identity may facilitate health-related behaviors via boosted self-efficacy. Self-efficacy is a person's conviction that they can successfully perform a particular behavior to achieve expected outcomes, and it has been shown to be a critical factor in performing health behaviors, particularly for challenging and complicated ones. While most research on factors contributing to the boost of self-efficacy concerned

intrinsic ones (e.g., health literacy), social identity, which is an extrinsic social factor, has also been found to exert significant influence on one's self-efficacy to perform (group-advocated) health behaviors. Based on social cognitive theory's emphasis on the role that model similarity plays in increasing self-efficacy, research has shown that social identity increases self-efficacy to perform in-group-advocated health behaviors due to a heightened sense of similarity to successful in-group models (Guan & So, 2016). This is because individuals tend to perceive in-group members as sharing various commonalities, which likely leads them to perceive other in-group members as similar to themselves. This sense of similarity a person perceives towards in-group members can boost their self-efficacy to engage in health-related behaviors advocated and successfully performed by other in-group members.

Social identity and health message effects

The capacity for the strength of social identity (i.e., identification with a group) to guide behaviors in terms of group membership indicates its utility as a strategy for health and risk messaging. When an individual identifies with a group (e.g., a work organization) in which a particular health-related behavior (e.g., vaccination) is advocated, prototypical, or normative (widely accepted and adopted by in-group members), they are highly likely to perform that behavior. Thus, a health message focusing on a narrative of an in-group member, who is performing and benefiting from an identity-based health-related behavior, may enhance the message effects – such as generating more favorable attitudes, stronger behavior intentions, and greater likelihood of message acceptance – compared with a message telling a story of an out-group member. Thus, it is useful to tailor the message in terms of salient identity and group membership of the intended audience. Of note, such effects depend on the extent to which one identifies with the in-group.

In addition, when people define themselves in terms of health-related conditions, choices, or behaviors (e.g., identification as a vegetarian), they are likely to maintain the behaviors and endorse the messages that emphasize their valued health identity. As health-related identities can be based on positive behaviors, they can also be related to negative ones. When an individual has an identity associated with an unhealthy behavior (e.g., identification as a smoker) and especially when the identity is salient, a great challenge for health communication is to persuade them to discontinue the behavior and dissociate from the group. Messages that associate an unhealthy behavior with an out-group may reduce the likelihood of in-group members performing that behavior. Messages that discourage an unhealthy behavior (e.g., anti-smoking messages) may also be perceived as a threat to one's identity, leading to message resistance and maladaptive responses, such as denial, counterarguing, and rationalization. Such effects are contingent upon the salience of the identity as well.

SEE ALSO: Intergroup; Message Tailoring; Social Cognitive Theory; Social Norms; Social Support.

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